INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Adjuvant trastuzumab emtansine (T-DM1)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00659
Version 2

T-DM1 by intravenous (IV) infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

Treatment stages

01 **T-DM1 after surgery**
14 cycles, every 3 weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

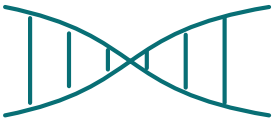
Your medicines and their different roles

After preoperative treatment and surgery, residual invasive HER2-positive cancer can lead to a change to T-DM1 to reduce recurrence risk.

HER2-DIRECTED ANTIBODY WITH A CHEMOTHERAPY PAYLOAD

Trastuzumab emtansine (T-DM1)

Kadcyla · antibody–drug conjugate



HER2 targeting carries DM1 into cancer cells.

The antibody attaches to HER2. After the complex enters a cell, its DM1 payload disrupts structures needed for cell division. This is a single linked medicine, with its own dosing and safety rules.

Attach to HER2 → Enter the cell → Release DM1 to disrupt division

What to watch for

Report bleeding, numbness, jaundice, new cough or breathlessness. Keep blood and heart checks.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

If invasive HER2-positive cancer remains in the breast or lymph nodes after the specified preoperative treatment, T-DM1 may be used after surgery to lower the chance of the cancer returning. The surgical pathology result is central to this decision.

Why not simply continue the earlier treatment?

Residual invasive disease changes the treatment discussion. Your team reviews the pathology, earlier medicines, recovery and other suitable options. This card describes a planned course of 14 postoperative T-DM1 cycles; it is not the same as the metastatic T-DM1 pathway.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

Your treatment aim

Indication

Adjuvant · residual invasive HER2-positive early breast cancer

Where this fits

T-DM1 by intravenous (IV) infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Each phase in your treatment plan

Adjuvant trastuzumab emtansine (T-DM1)

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

14 CYCLES, EVERY 3 WEEKS

T-DM1 after surgery

14 cycles, every 3 weeks

These are 14 postoperative cycles. Do not subtract earlier HER2 doses to make the course end at one year from the original diagnosis.

The order depends on the indication and your recovery. The whole-treatment map shows how local treatment and ongoing medicines relate to each other.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle numbers are examples for uninterrupted three-weekly treatment. Schedule checks by elapsed time and clinical need. Moving to maintenance does not restart the cardiac-monitoring clock. The clinical team books and reviews results; this map does not track completed tests.

Check	Before this phase	Before every 21-day cycle	Week 12 · around C5	Weeks 24 / 36 · around C9 / C13	Later / treatment change
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline cardiac assessment and left ventricular ejection fraction (heart pumping measurement) (LVEF) before T-DM1	Review symptoms; earlier echo if clinically indicated	Echo / MUGA at 12 weeks, around C5	Repeat around C9 and C13 if uninterrupted	Review at completion; agree follow-up based on cardiac risk and findings.
Blood tests	Baseline full blood count (FBC), renal and liver profile	FBC, renal and liver tests before every cycle	Continue pre-cycle blood tests	Continue pre-cycle blood tests	Follow abnormalities and the next treatment plan

Check	Before this phase	Before every 21-day cycle	Week 12 · around C5	Weeks 24 / 36 · around C9 / C13	Later / treatment change
Cancer imaging / markers	Surgery and pathology establish the indication	Review new symptoms at every visit	No routine computed tomography (CT) series if well in early breast follow-up	Agree breast surveillance and any symptom-led tests	Annual mammography of remaining breast tissue; additional imaging if indicated
electrocardiogram (ECG) / bone health	ECG when indicated by cardiac assessment; dual-energy X-ray absorptiometry (bone density) (DEXA) only if endocrine / fracture risk indicates	No automatic every-cycle ECG or DEXA	Risk-based clinical plan	Risk-based clinical plan	Keep any separately prescribed endocrine and bone-health follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00659 v2 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Adjuvant trastuzumab emtansine (T-DM1): the whole pathway

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before surgery	Surgery / pathology	T-DM1 · 14 cycles	Completion	Follow-up
Cancer medicines NCCP 00659	Prior taxane + HER2 treatment	Confirm residual invasive disease	T-DM1 every 21 days	Review final dose / unresolved effects	Endocrine therapy continues if indicated
Surgery EARLY	Neoadjuvant treatment precedes operation	Lumpectomy / mastectomy and node surgery	Wound and arm rehabilitation	Review local symptoms	Long-term local surveillance

Care / check	Before surgery	Surgery / pathology	T-DM1 · 14 cycles	Completion	Follow-up
Radiotherapy EARLY	multidisciplinary team (MDT) assesses likely need	Pathology determines indication	radiotherapy (RT) may overlap T-DM1 under MDT plan	Report cough / breathlessness	Assess possible late lung / cardiac effects
Blood tests NCCP 00659	Prior-treatment checks	Baseline FBC, liver and renal tests	FBC including platelets, renal and liver tests before every cycle	Review abnormal results	As required by continuing care
Heart checks NCCP 00659 · ESC	Baseline / earlier HER2 monitoring	Review LVEF before switching	Every 12 weeks: around cycles 5, 9 and 13 if uninterrupted; earlier if needed	Completion assessment	Risk-based surveillance
Cancer imaging EARLY · BONE	Response imaging for surgery	Pathology establishes this indication	No routine CT series if well	Agree mammogram schedule	Annual mammography of remaining breast tissue

What each check is for

1

Blood tests and treatment review

Prior-treatment checks → Baseline FBC, liver and renal tests → Before every cycle: platelets, liver and renal tests → Review abnormal results → As required by continuing care

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

2

Heart tests

Baseline / earlier HER2 monitoring → Review LVEF before switching → Every 12 weeks; sooner if abnormal / symptoms → Completion assessment → Risk-based surveillance

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

3

Imaging and markers

Response imaging for surgery → Pathology establishes this indication → No routine CT series if well → Agree mammogram schedule → Annual mammography of remaining breast tissue

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

4

Bone health with endocrine treatment

At endocrine / ovarian-suppression planning; repeat by risk

Low oestrogen can affect bone strength.

Ask / act: Ask whether DEXA, fracture-risk assessment or bone protection is appropriate.

5

Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the first visit

Blood tests, cardiac assessment, medicines review and the consent conversation come before T-DM1. Confirm recovery from surgery and the pathology-based treatment plan.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Trained staff check and access the line, give the prescribed premedicines and infusions, and monitor for reactions. Tell the nurse immediately about pain, burning, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

The first T-DM1 infusion takes 90 minutes, followed by at least 90 minutes of observation. Later tolerated infusions can take 30 minutes, followed by at least 30 minutes of observation. These are infusion and observation times, not the whole visit.

Before starting and between visits

Plan the tests, access and support before the first dose.

Blood tests

Prior-treatment checks → Baseline FBC, liver and renal tests → Before every cycle: platelets, liver and renal tests → Review abnormal results → As required by continuing care

Heart checks

Baseline / earlier HER2 monitoring → Review LVEF before switching → Every 12 weeks; sooner if abnormal / symptoms → Completion assessment → Risk-based surveillance

Imaging, local treatment and follow-up

The whole pathway has several connected parts.

Scans and imaging

Response imaging for surgery → Pathology establishes this indication → No routine CT series if well → Agree mammogram schedule → Annual mammography of remaining breast tissue

Radiotherapy relationship

MDT assesses likely need → Pathology determines indication → RT may overlap T-DM1 under MDT plan → Report cough / breathlessness → Assess possible late lung / cardiac effects

Bone health

If an aromatase inhibitor or ovarian suppression is prescribed, assess fracture risk and baseline DEXA as indicated. Repeat timing depends on the result, risk and bone treatment; it is not an automatic scan for everyone.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
T-DM1 infusion	 **Day 1** Planned dose after review
Infusion reaction	 **Day 1** During / after infusion; report promptly
Bruising / bleeding	 **Days 1–21** Report changes; keep platelet checks
Heart / breathing symptoms	 **Days 1–21** Seek advice on any day
Numbness / tingling	 **Days 1–21** Report before the next dose

Treatment bands: Show the planned medicine schedule, subject to review and dose changes.

Checks and restart: Keep the arranged blood / heart appointments. A calendar day alone does not authorise the next dose.

Symptoms: Can occur at any time; broad bands are not measured risk curves.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00659 v2 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

Between-treatment notes

Day 1–21: Your reference cycle

T-DM1 by IV infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change. Check the agreed prescription and contact the team for new symptoms. Restart only after the team confirms the plan.

Support around the medicines

1 **Support for this phase**

T-DM1 is low emetogenic risk. Confirm the prescribed anti-sickness plan and infusion-reaction instructions; check the full drug name.

2 **Bloods and cardiac checks**

Baseline FBC, renal and liver tests and cardiac assessment before T-DM1. Repeat blood tests before every cycle. Echo/MUGA every 12 weeks, around cycles 5, 9 and 13 if uninterrupted; earlier if symptoms or abnormal results require it.

3 **Visits and practical support**

Ask about travel, work, fertility, menopause, exercise and help at home. The team arranges and reviews clinical checks; the patient should not have to calculate the monitoring timetable.

Questions worth asking

When are my next tests?

Your team should confirm the next test, who books it, and who reviews the result before treatment. The map shows expected intervals; it does not track completed tests.

Does every scan follow a fixed schedule?

Annual mammography of remaining breast tissue. New symptoms can require earlier assessment.

Why was this treatment chosen for me?

If invasive HER2-positive cancer remains in the breast or lymph nodes after the specified preoperative treatment, T-DM1 may be used after surgery to lower the chance of the cancer returning. The surgical pathology result is central to this decision. Residual invasive disease changes the treatment discussion. Your team reviews the pathology, earlier medicines, recovery and other suitable options. This card describes a planned course of 14 postoperative T-DM1 cycles; it is not the same as the metastatic T-DM1 pathway.

Try explaining the plan back: Which medicines are you taking now? What changes in the next phase? Which checks are due and who reviews them before treatment?

Trastuzumab emtansine (T-DM1)

- Low platelets and bleeding
- Liver injury, rarely serious
- Numbness or tingling
- Heart pumping-function reduction
- Rare serious lung inflammation
- Fatigue, nausea and infusion reactions

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant T-DM1

Active medicines: Trastuzumab emtansine

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Premedicines are not usually required. Prior infusion reactions need individual review.
- Check platelets, liver results and cardiac status. Verify T-DM1, not trastuzumab alone or T-DXd.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Adjuvant trastuzumab emtansine (T-DM1)

T-DM1 by intravenous (IV) infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Low platelets / bleeding

T-DM1

Keep platelet checks. New bruising or bleeding needs advice; heavy or persistent bleeding needs urgent help.

3 Heart pumping function

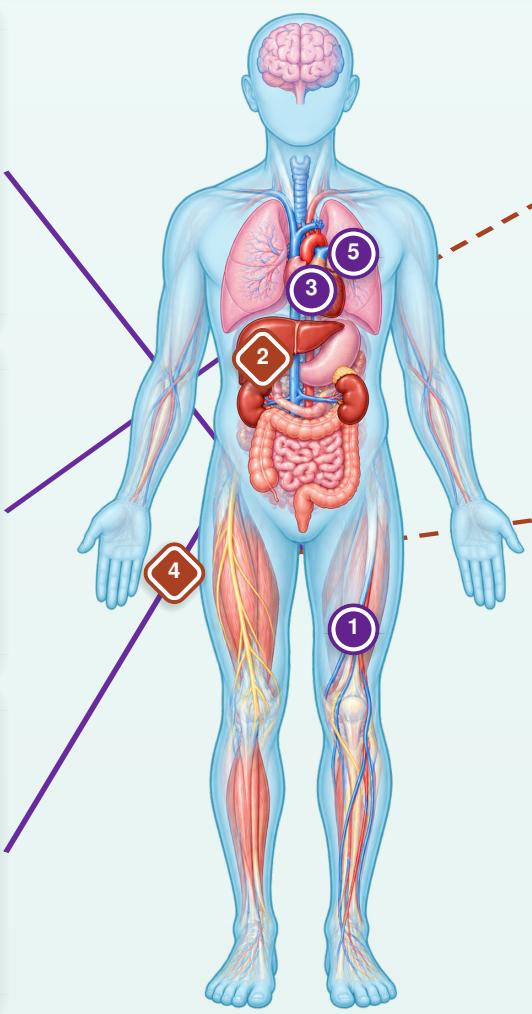
T-DM1

Keep echoes. Report new breathlessness, swelling or reduced exercise tolerance.

5 Lung inflammation

T-DM1 / radiotherapy context

New cough or breathlessness needs assessment, particularly if radiotherapy overlaps.



2 Liver injury

T-DM1

Liver tests are checked before each cycle. Report yellow skin / eyes or dark urine.

4 Peripheral nerves

T-DM1

Report numbness, tingling or difficulty with daily tasks before the next dose.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00659 v2 · official regimen**
   https://healthservice.hse.ie/documents/6680/659_v2_Trastuzumab_Emtansine_EBC_21days.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
6. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
7. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NCCP NCIS antiemetics v8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf